

CITY CARE HOSPITAL

123 Health Boulevard, Andheri West, Mumbai - 400058 | NABH Accredited

DISCHARGE SUMMARY

Patient Name:	Ankit Sharma	Age / Sex:	45 Years / Male
MRN / Patient ID:	CCH-IP-2025-1247	Bill Number:	CCH/2025/03/4821
Admission Date:	11/03/2025	Discharge Date:	18/03/2025
Length of Stay:	7 days	Ward(s):	ICU (3d), Private Deluxe (4d)
Consultant:	Dr. Rajesh Mehta, MD DM (Cardiology)		
Attending Surgeon:	Dr. Vikram Singh, MS MCh (CTVS)		

FINAL DIAGNOSIS

Primary: Acute Coronary Syndrome (ACS) with Triple Vessel Coronary Artery Disease (CAD-TVD). ICD-10: I25.10

Secondary:

- Type 2 Diabetes Mellitus, controlled (ICD-10: E11.9)
- Essential Hypertension, controlled (ICD-10: I10)
- Hyperlipidemia (ICD-10: E78.5)

PROCEDURES PERFORMED

1. Coronary Angiography (Pre-operative assessment)

Date: 11/03/2025 | Performed by: Dr. Rajesh Mehta

2. Emergency Coronary Artery Bypass Grafting (CABG x 3 vessels)

Date: 12/03/2025 | Performed by: Dr. Vikram Singh

Grafts: LIMA-LAD, SVG-OM, SVG-PDA

Drug-Eluting Stent (DES) was placed during the procedure.

3. Echocardiography (2D Echo with Color Doppler)

Date: 12/03/2025 | LVEF: 45% post-procedure

CLINICAL COURSE / HOSPITAL STAY

Patient presented to the Emergency Department on 11/03/2025 with severe central chest pain radiating to the left arm, diaphoresis, and shortness of breath. ECG revealed ST-segment elevation in leads V1-V4 suggestive of acute anterior wall myocardial infarction. Troponin-I was elevated at 8.4 ng/mL. Patient was immediately admitted and taken up for emergency coronary angiography which revealed triple vessel disease with critical stenosis in LAD (95%), LCX (80%), and RCA (75%).

Patient underwent emergency CABG x 3 vessels on 12/03/2025. The procedure was uneventful. Post-operatively, patient was shifted to the Cardiac ICU where he remained for 3 days (12/03 to 14/03) on monitoring with vasopressor support which was gradually tapered. Patient was then shifted to a private deluxe room on 15/03/2025 and remained there until discharge on 18/03/2025.

During the post-operative period, patient was managed with intravenous Heparin (5000 IU x 12 vials over 3 days), IV Pantoprazole 40mg for stress ulcer prophylaxis, IV Paracetamol 1g for pain management, and oral Atorvastatin 40mg from POD 2 onwards. Patient tolerated medications well.

DISCHARGE MEDICATIONS

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|----|-----------------------|--------------------------|-----------|
| 1. | Tab. Aspirin 75mg | Once daily, after dinner | Lifelong |
| 2. | Tab. Clopidogrel 75mg | Once daily, morning | 12 months |

3.	Tab. Atorvastatin 40mg	Once daily, at bedtime	Lifelong
4.	Tab. Metoprolol 25mg	Twice daily	Review at 1 month
5.	Tab. Pantoprazole 40mg	Once daily, before breakfast	1 month
6.	Tab. Metformin 500mg	Twice daily, with meals	Continue

CONDITION AT DISCHARGE

Patient is hemodynamically stable, ambulatory, and tolerating oral feeds. Vital signs: BP 130/80 mmHg, PR 78/min, SpO2 98% on room air, afebrile. Surgical site is clean and dry with no signs of infection. Patient and family have been counselled regarding lifestyle modifications and follow-up.

FOLLOW-UP INSTRUCTIONS

- OPD review with Dr. Vikram Singh in 7 days (25/03/2025)
- Echocardiography after 1 month
- Lipid profile, HbA1c, and renal function after 3 months
- Emergency contact: +91-22-2674-XXXX (24x7)

Dr. Rajesh Mehta, MD DM (Cardiology)
Consultant Cardiologist, City Care Hospital
Date: 18/03/2025